DERMATOLOGY OSCE SLIDES

Bullous impetigo



Infected umbilical stump



Crusted impetigo



Infantile atopic dermatitis (secondary impetiginization)

gambar slide prof adalah y face area, mata pon termasuk jugak



Subacute atopic dermatitis



Lip licking dermatitis (irritant contact dermatitis)



ichthyosis





Allergy contact dermatitis (eg: nikel)





Lichen planus



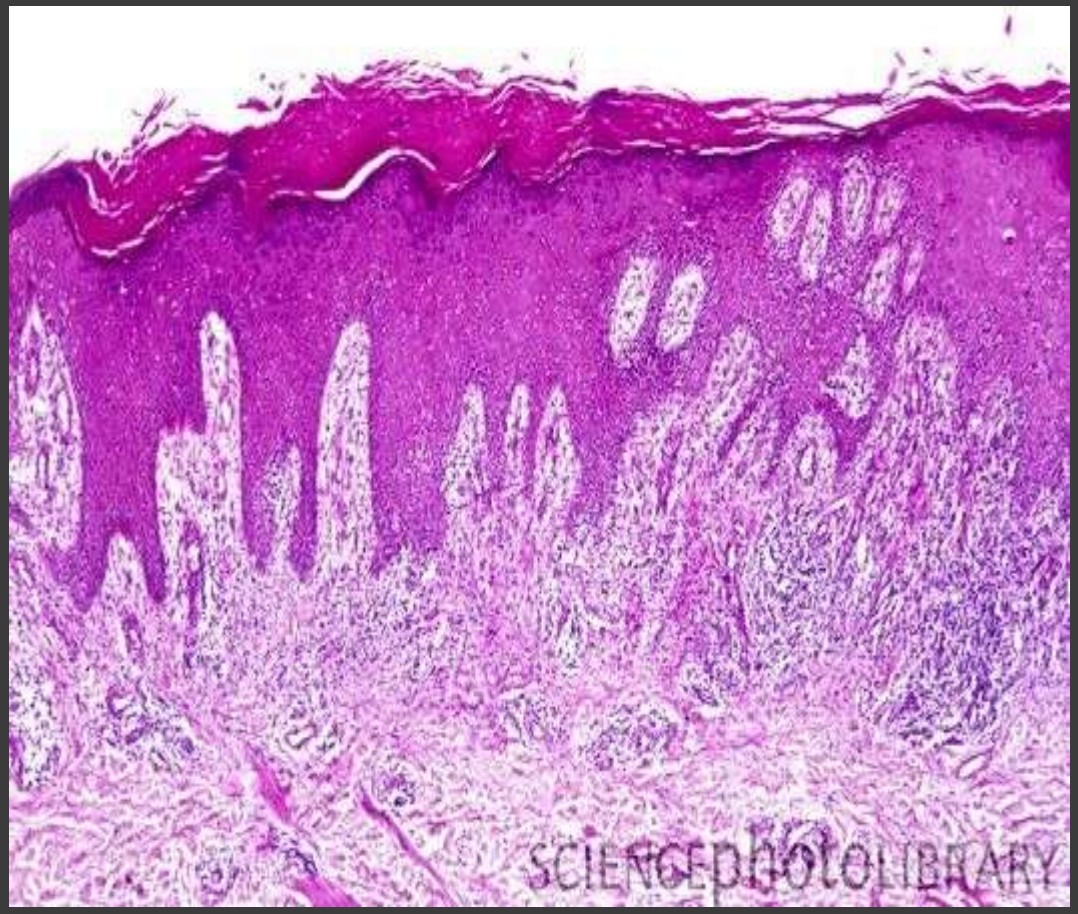


Lichen planopiliaris (ddx: discoid lupus)





Lichen planus histology section (saw teeth acanthosis)





Urticaria wheal + dermographism







Mucosal lichen planus





Urticaria (extensive)





angioedema

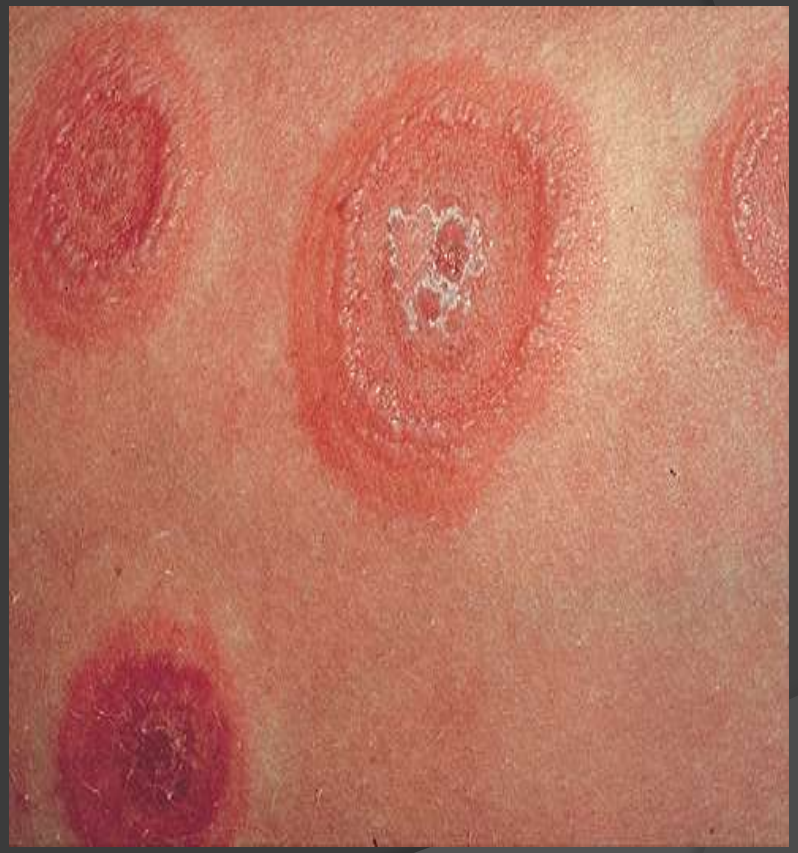



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Erythema multiforme





Xeroderma pigmentosa (TAK MASUK exam)



Pediculosis capitis (notice the nits)



Pityriasis alba

* patch
*residual erythema
*atopic infant



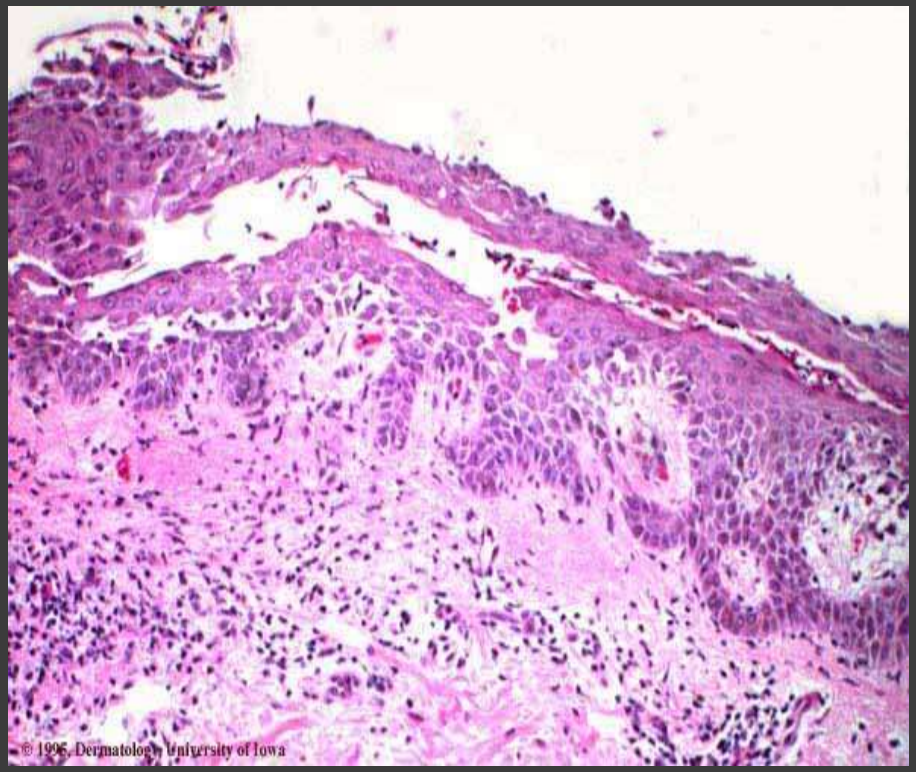


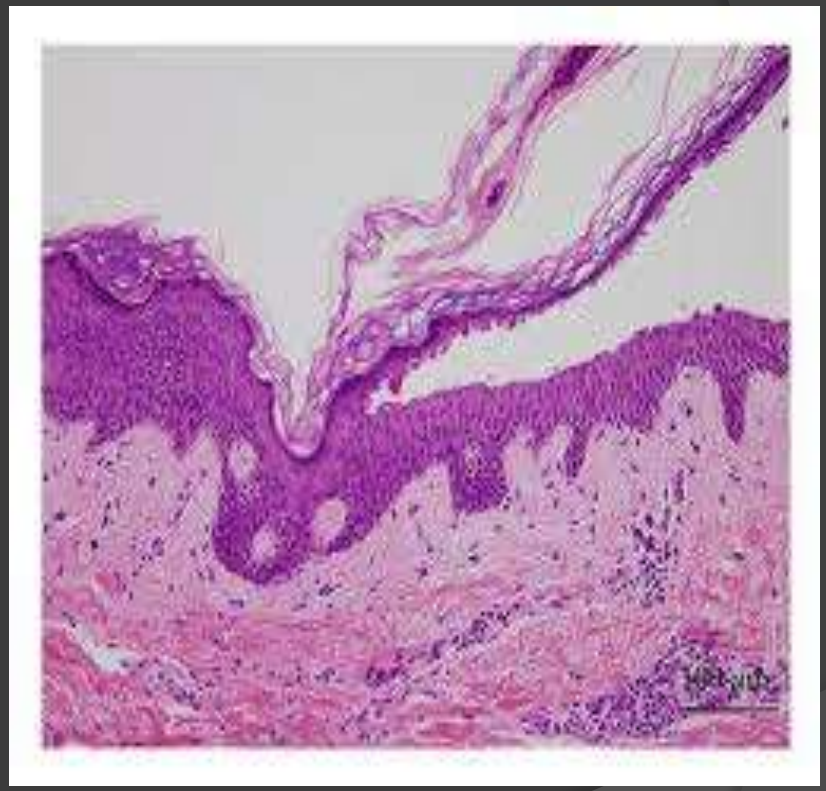
Gravitational eczema



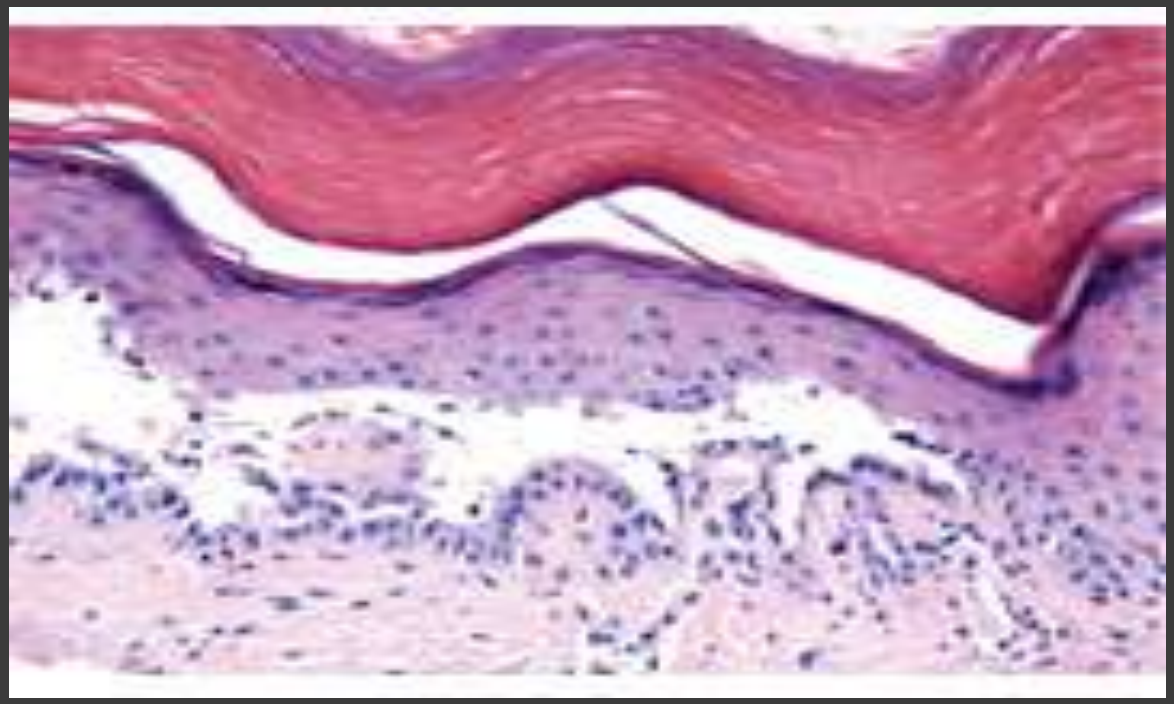


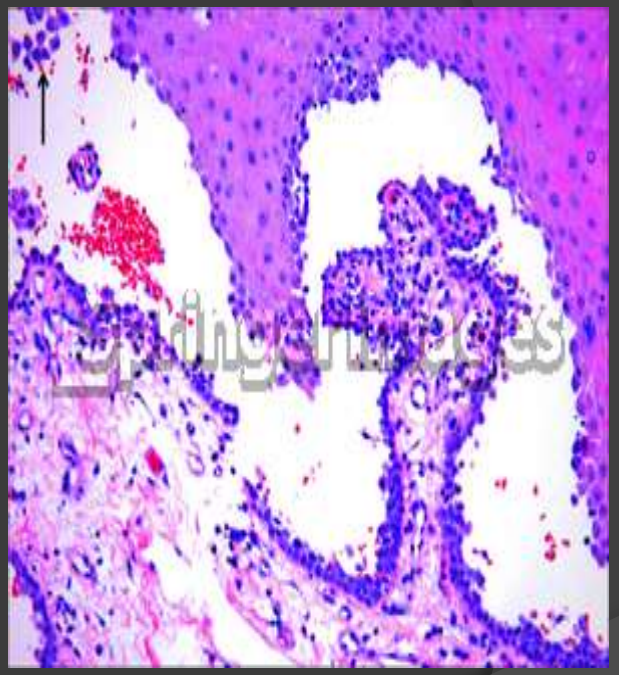
Pemphigous foliaceus (subcorneal acantholytic bullae)



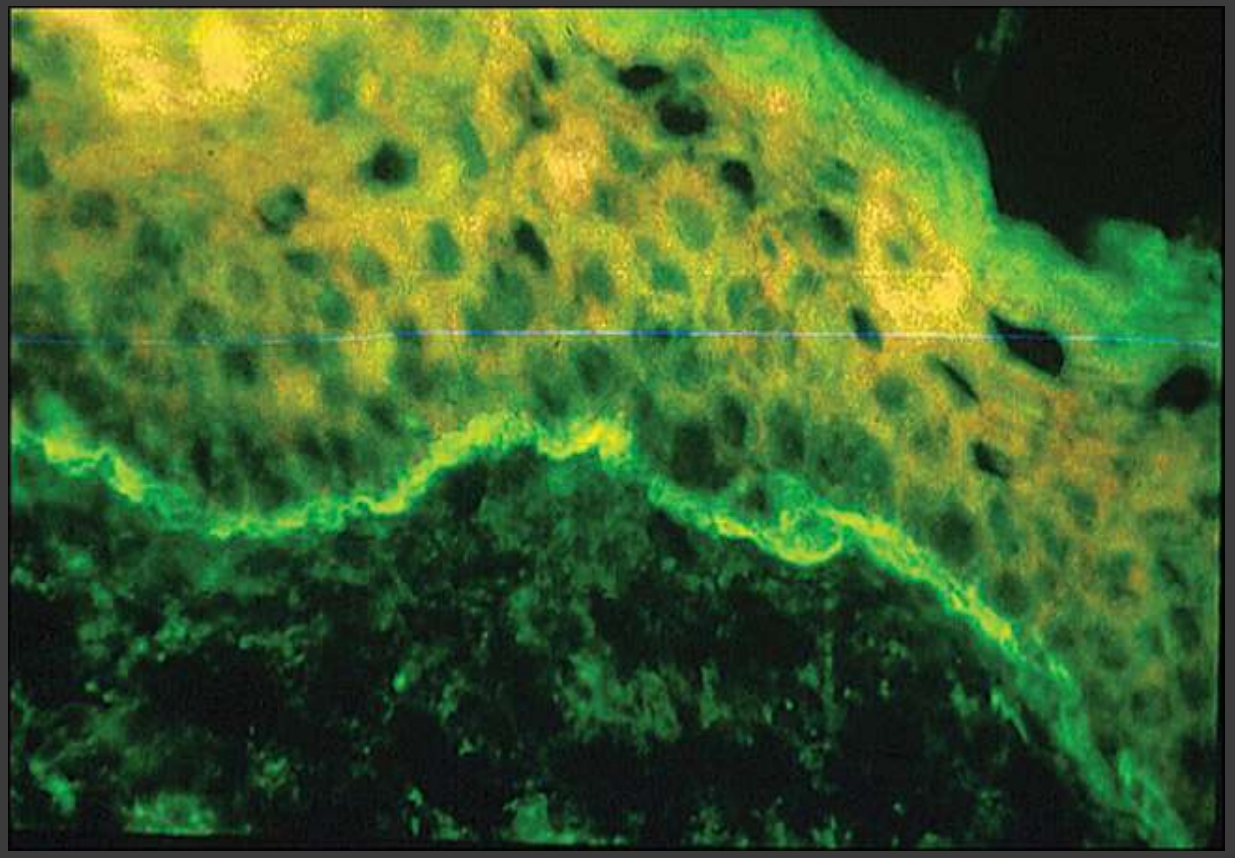


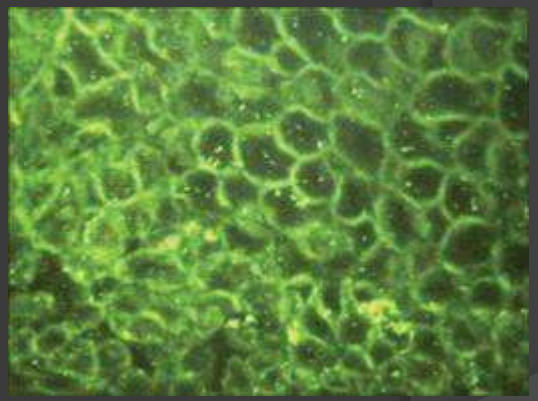
Pemphigous vulgaris (suprabasal acatholytic bullae)





Pemphigous (direct immunoflourescent)





- END OF FIRST PART REVISION SLIDE

psoriasis






http://derm.net





Acute psoriasis



Lichen planus





Herald patch





Local vitiligo –acrofacial vitiligo (several foci on one site)



Acne vulgaris






SFS

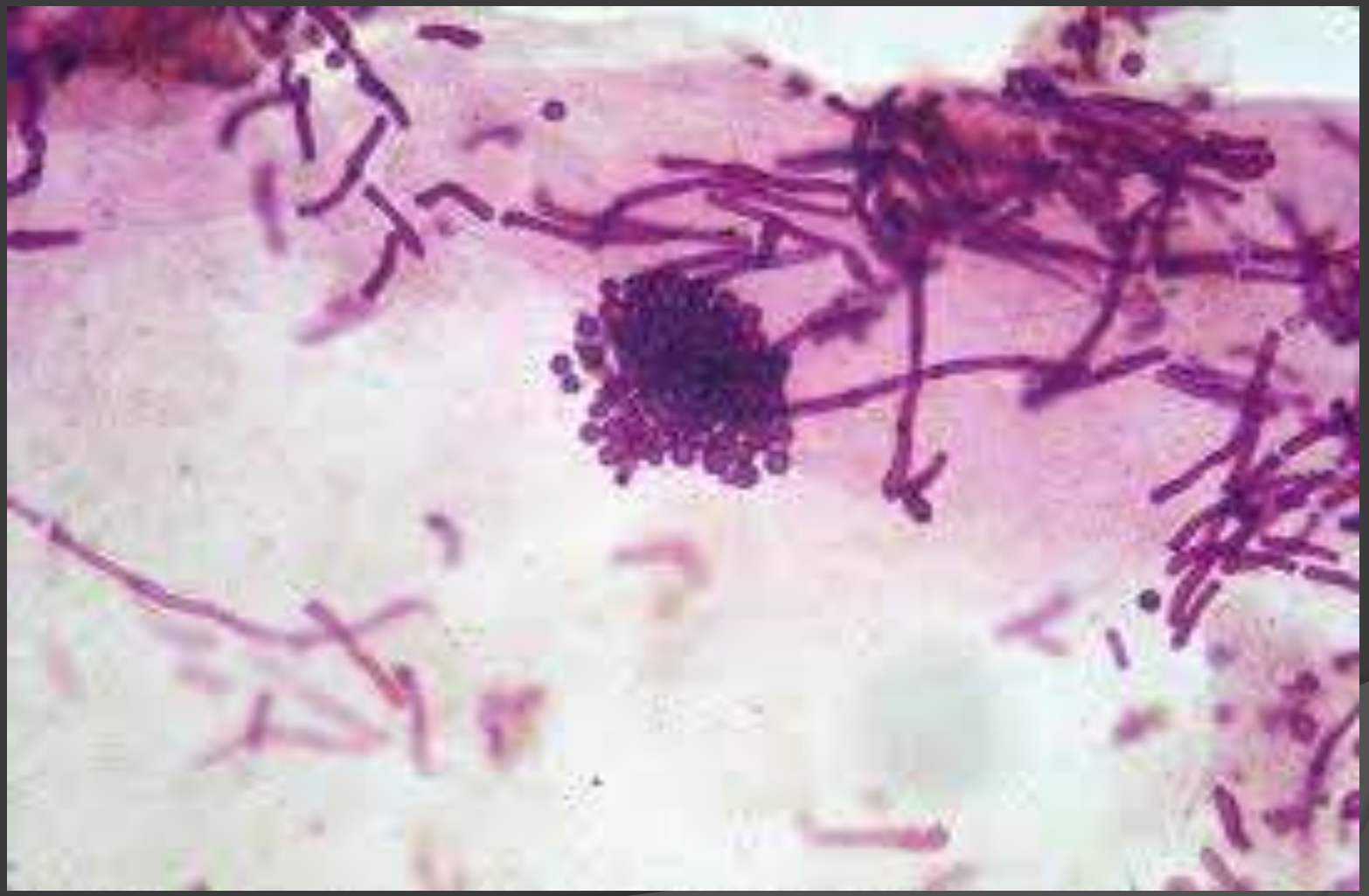

Whitehead
Blackhead
Papule
Pustule

Pityriasis versicolor (hypopigmented/hyperpigmented)





Dimorphic feature of tinea versicolor



Toe web maceration





Tinea pedis

- Types:
- Interdigital
- Erythematous
- Vesiculobullous
- hyperkeratosis



Tinea mannum (types, as tinea pedis)



Tinea circinata



Tinea circinata



Non inflammatory Tinea capitis

grey patch / black dot

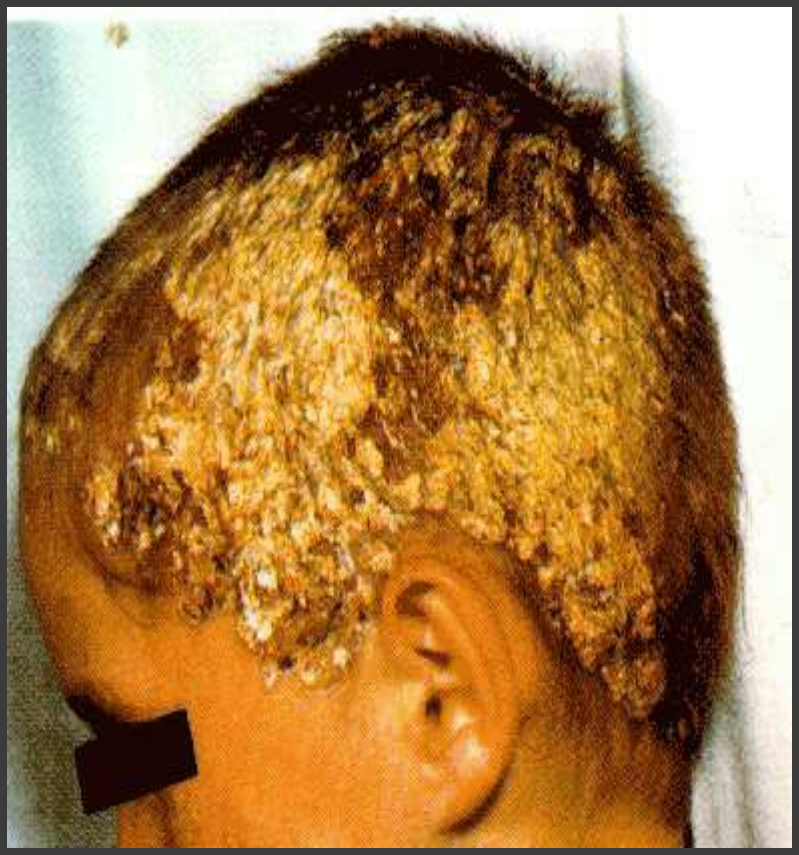




Inflammatory tinea capitis (kerion celzi)



favus





Nail dystrophy (eg: onychomycosis) -causes?





erythrasma







onychomycosis



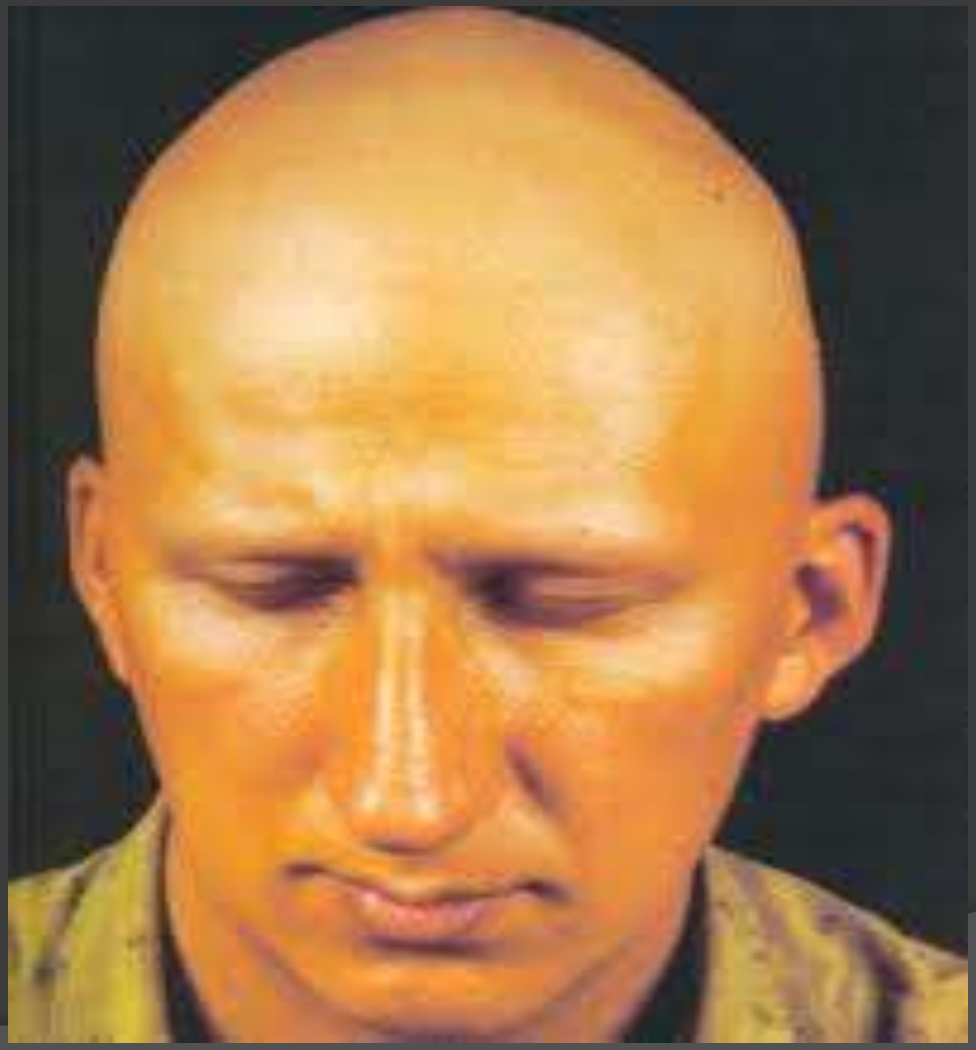


Alopecia areata
(Q= sign of bad prognosis)





Alopecia areata universalis
(hint : no eyebrow)

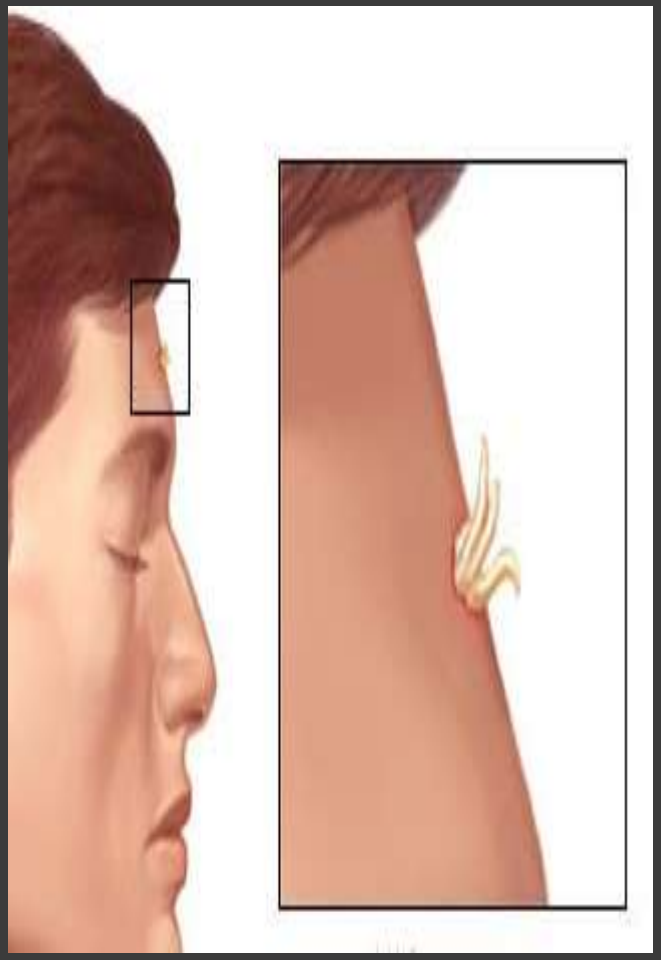


Verrucae vulgaris





Verrucae filiformis





Verrucae digitiform



Verrucae plana



Molluscum cantagiosum





Herpes simplex





Herpes genitalis





Herpes zoster





Chicken pox





Condyloma accuminata



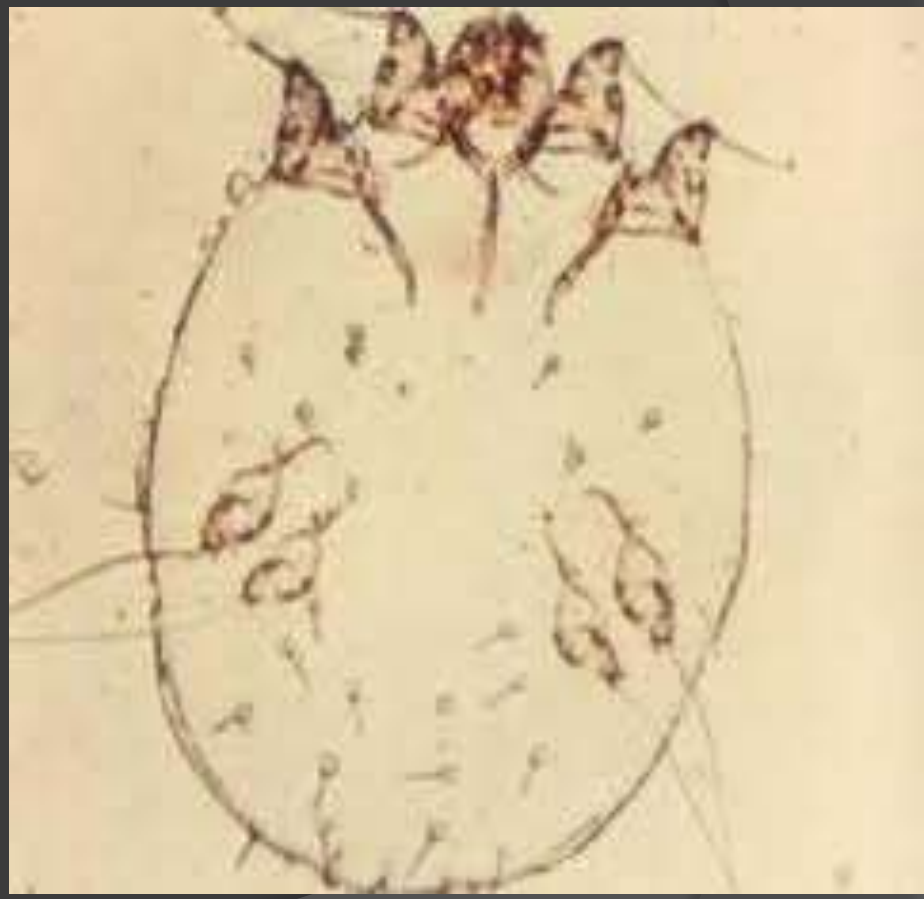


Condyloma lata



scabies





Phototherapy (Q=indication)



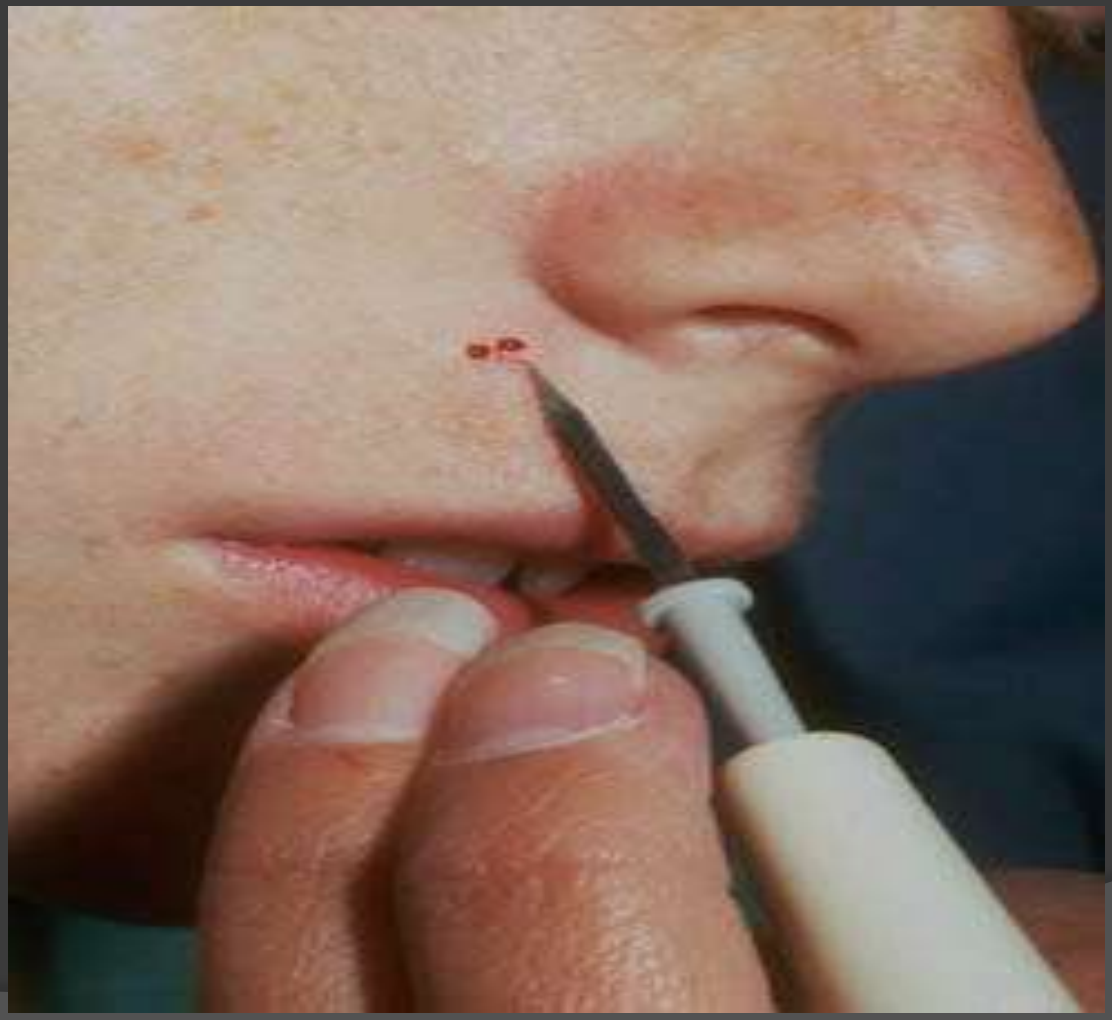


UVB

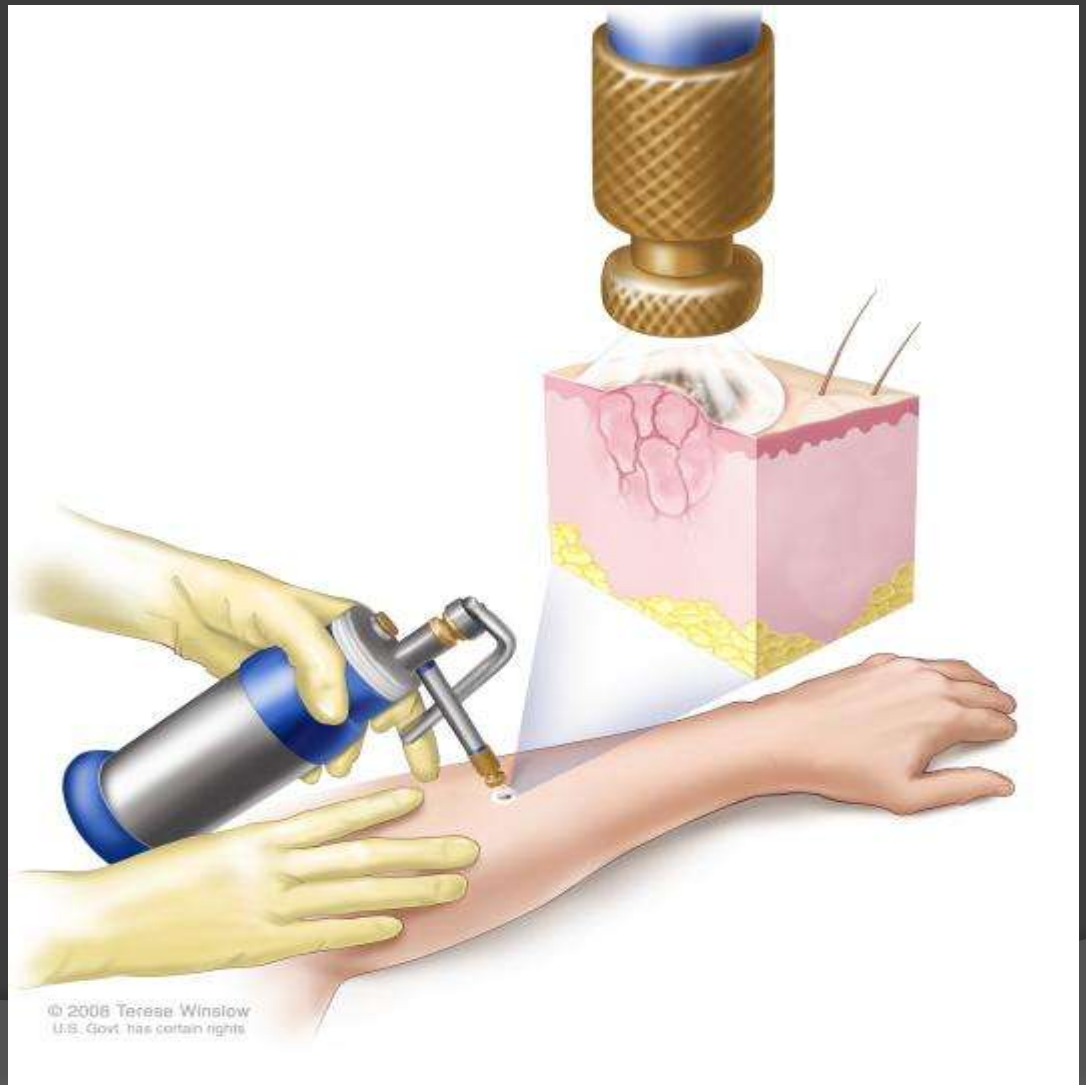




electrocautery



cryotherapy





laser



woodlight



Dermatology Revision

Contents

- The basics – history and examination
- Two cases
- Melanoma
- Acne
- Picture quiz

The basics - History

- **Duration**
  - Onset – when, sudden/gradual, acute/ongoing
  - Where it started, how it started, change over time
  - Previous episodes

- **Location**
  - Flexural/extensor surface, sun-exposed areas, ‘sweaty’ areas

- **Associated symptoms**
  - Itch, tender, bleeding, discharge, pain, swelling
  - Systemic features - pyrexia, joint pain, weight loss, malaise

- **Response to treatment, aggravating and relieving factors**
  - OTC, prescribed medications, herbal

- PMHx
- DHx
  - New medications, OTC, allergies
- FHx
- Occupation, hobbies
  - Exposure to chemicals
- Travel
- SHx, social sequelae

Examination

- Describing lesions
  - Generalised/localised/symmetrical
  - Size, shape, site, colour, well or ill-defined, morphology
  - Nails, mucosa
  - ABCDE

•CASE 1

- Jane is a 5 year old girl who is brought to see you, the GP, with an itchy rash



History

- Jane is 5 years old
- Itchy rash on insides of elbows, backs of knees and eyelids
- Sometimes bleeds with scratching
- Started on the elbows but has gotten bigger and worse
- Worse in hayfever season and with pets
- Had something similar as a baby on her face, cleared up with creams that the doctor gave her
- PMHx of hayfever
- FHx of eczema and asthma
- No DHx, no exposure, no travel or contacts

Eczema

- Defect in skin barrier, water is lost and skin sensitivity is increased, skin becomes inflamed and itchy
- Eczema = dermatitis

Intrinsic
- Atopic
- Seborrhoeic
- Pompholyx
- Discoid
- Venous

Extrinsic
- Contact allergen dermatitis
- Contact irritant dermatitis

Atopic eczema

- Common, children, Hx or FHx of atopy
- Chronic and relapsing – ‘flares’
- Triggers
  - Stress, heat, soap, pets
- Acute or chronic
  - Wet, oedematous
  - Dry, lichenification
- Children
  - Facial and flexures
- Adults
  - Flexures and hands



Differentials

- Scabies
- Seborrhoeic dermatitis
- Contact dermatitis
- Drug eruption

Scabies



Seborrhoeic dermatitis



Contact dermatitis



Investigations and Management

- Clinical diagnosis

- Emollients and soap substitutes
- Topical steroids – mild to potent ladder – FingerTip Unit – short courses only
- Avoid triggers
- Wet wraps

- Anti-histamines, calcineurin inhibitors, phototherapy, immunosuppressants
- Clinical psychologist, Dermatology nurse

Complications

- Secondary bacterial infection
  - Impetigo
- Secondary viral infection
  - Eczema herpeticum
  - Molloscum contagiosum

Impetigo



Eczema herpeticum

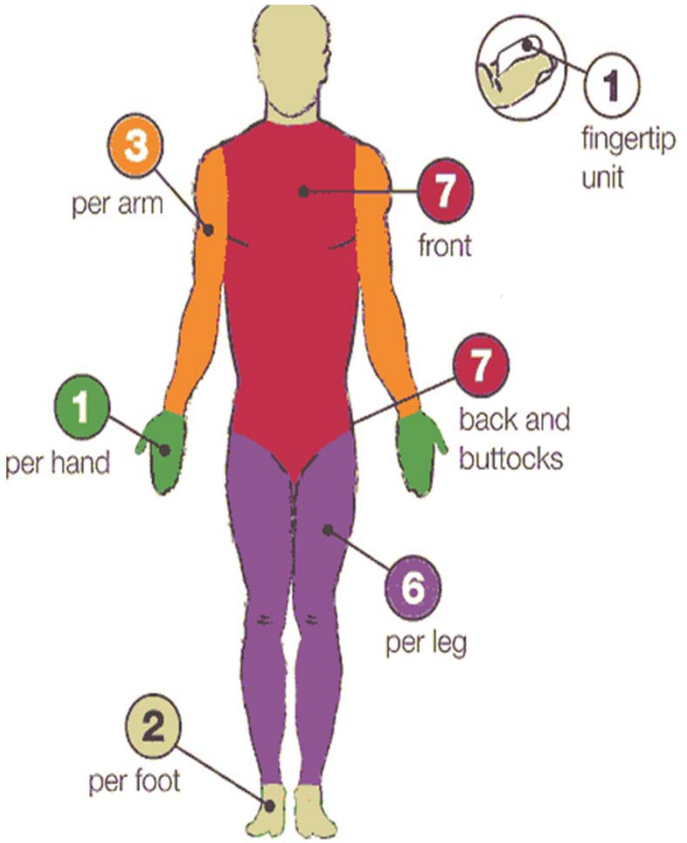


Molloscum contagiosum



…My son has been diagnosed with eczema, can you explain the treatment?

- Emollients and soap substitutes used to replace skin barrier
- Topical steroids - FingerTip Unit
- Avoid triggers
- Wet wraps, dermatology nurse
- Other treatments available if severe



- Warn about coldsores and impetigo
- Offer take-home information and review appointment, opportunity to ask questions

- Patch testing? Dangers of steroids? Can it be cured? Why do they have it?

•CASE 2

- John, a 16 year old male, comes to the GP with an itchy rash on his knees



History

- **Duration**
  - Onset – when, sudden/gradual, acute/ongoing
  - Where it started, how it started, change over time
  - Previous episodes
- **Location**
  - Flexural/extensor surface, sun-exposed areas, ‘sweaty’ areas
- **Associated symptoms**
  - Itch, tender, bleeding, discharge, pain, swelling
  - Systemic features - pyrexia, joint pain, weight loss, malaise
  - Nail changes, scalp involvement, eyes
  - IBD, Metabolic syndome
- **Response to treatment, aggravating and relieving factors**
  - OTC, prescribed medications, herbal

History

- Past medical history
  - Asthma, hayfever, allergies

- Drug history
- Family History
- Exposure
  - Chemicals, pets, cosmetics
- Travel
- Contacts

History

- John is 16 years old
- Itchy red rash that flakes on the outsides of his knees and elbows, and scalp
- Started on his elbows but has gotten worse
- No other systemic features
- He’s been stressed at school with his GCSEs and recently started smoking
- FHx of psoriasis and arthritis
- No DHx, exposures, travel or contacts

Psoriasis

- Chronic inflammatory skin condition
- Increased skin and keratin production
- FHx, associated conditions
- Triggers – alcohol, smoking, stress, trauma, medications

Types
Chronic Plaque
Erythrodermic
Guttate
Pustular
Palmo-plantar
Flexural

Chronic Plaque Psoriasis

- Symmetrical, well defined pink plaques, thick silvery scale, extensor surfaces
- Nail pitting, onycholysis
- Psoriatic Arthritis







Differential diagnoses

- Bowens disease
- Seborrhoeic dermatitis
- Discoid eczema
- Tinea corporis
- Drug eruption

Bowens disease



Discoid eczema



Tinea corporis



Investigations and Management

- Topical
  - Emollients and soap substitutes
  - Emollients + salicyclic acid
  - Topical steroids
  - Tar preparations
  - Vitamin D analogues

- Severe
  - Phototherapy UVB or PUVA
  - Acitretin (Retinoid, Vitamin A analogue)
  - Cyclosporin / Methotrexate
  - Biologics e.g. infliximab, adalimumab

Psoriasis…some OSCE points

- UVB – good but limit exposure

- Arthropathy – up to 30%
- Increased risk of metabolic syndrome
- Quality of life

- Chronic condition, not infectious, doesn’t cause scarring
- Warn about triggers
- Familial link – may occur in family members/children

Melanoma History

- Presenting complaint
- History of presenting complaint:
  - Timescale
  - Bleeding
  - Crusting/oozing
  - Itch
  - Other skin lesions
- Past Medical history
  - Previous skin lesions
  - Autoimmune conditions
- Risk factors
  - Burns easily, fair skin
  - Previous sun burns
  - Time spend abroad
  - Sun bed use
  - Family history
  - Medications
  - Occupation/hobbies

Melanoma Lesion

- ABCDE assessment
- Asymmetry
- Border irregularity
- Colour – two or more
- Diameter >6mm
- Evolution over time

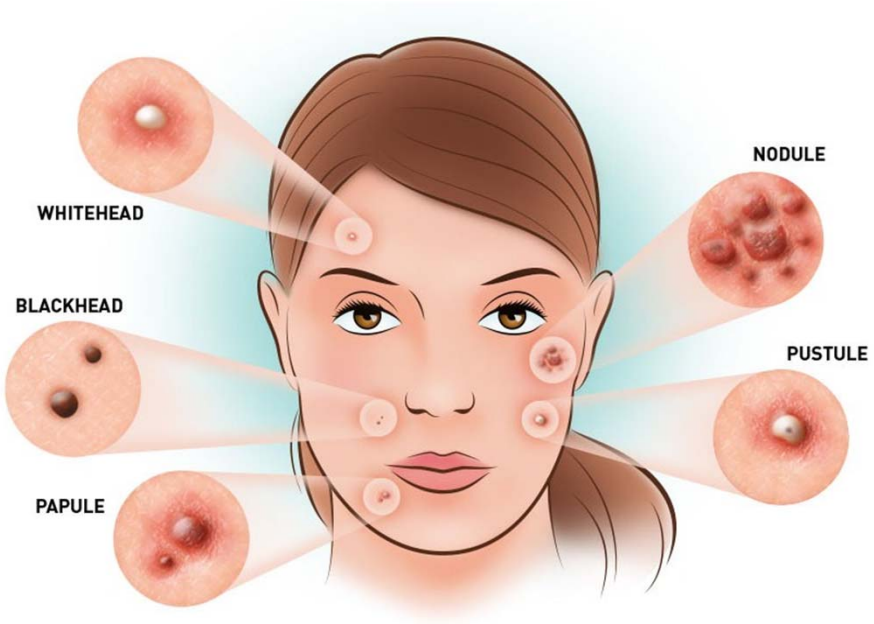


Melanoma Advice

- Avoid sunburn
- Sun protection – UVA and UVB suncream, clothing, hats
- Avoid sun in the middle of the day
- Monitor moles
- Don’t ever use sunbeds

Acne

- What is it?
  - Chronic inflammation of glands, increased sebum production
  - Most clear by early 20s, affects 80% of teenagers
- Treatment?
  - Antiseptic face wash
  - Topical retinoids
  - Topical antibiotics
  - Oral antibiotics
  - OCP
  - Isotretinoin – specialist only
    - Dry skin, avoid alcohol, mood changes, teratoge



PICTURE QUIZ

- Painful red swollen leg
- Feverish

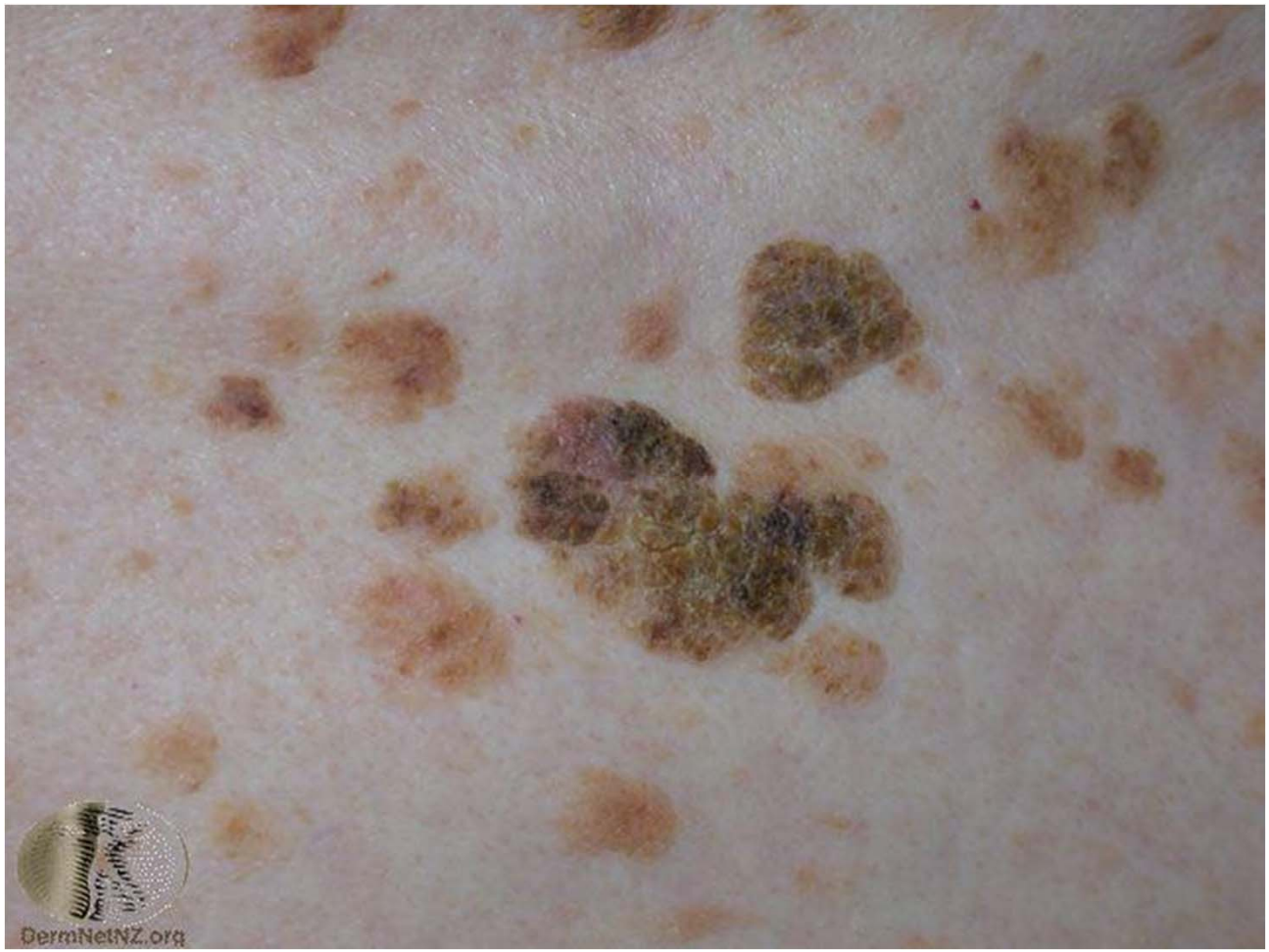


- Sudden onset of fever, chills and facial rash
- Well defined, raised border
- Blistered and painful





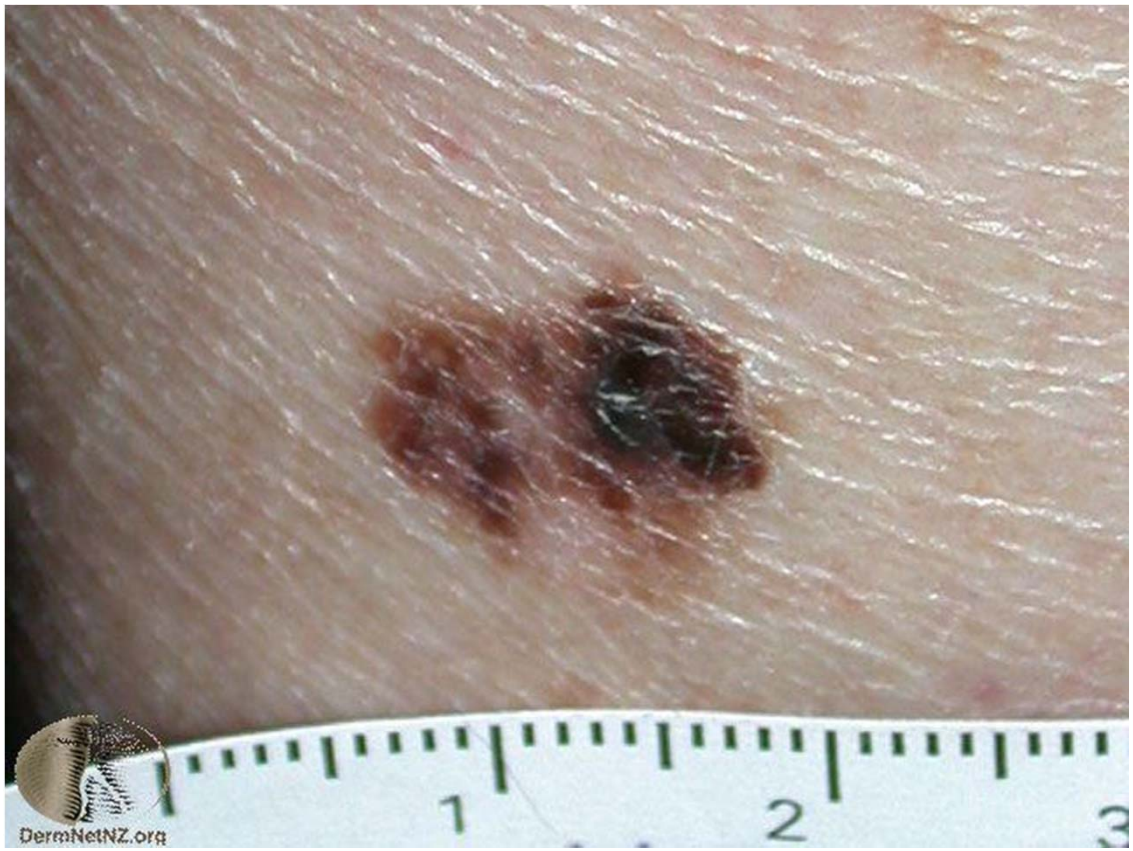
- 70 year old man



- 70 year old man
- On forehead



- Change in a pre-existing mole




DermNetNZ.org


DermNetNZ.org

- 30 year old female with a sore throat



- Developed a cold sore recently



- Itchy rash



- 4 year old boy
- High spiking fever
- Red conjunctiva
- Skin peeling on fingertips
- Cervical lymphadenopathy



- Flu like illness after hiking in the woods one week ago




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- Started as a small blister, rapidly progressed
- PMHx IBD



- PMHx diabetes





- Currently taking penicillin

